

## **WHO advice**

### **Surveillance**

Due to the constantly evolving nature of influenza viruses, WHO continues to stress the importance of year-round global surveillance to detect and monitor virological, epidemiological and clinical changes associated with emerging or circulating influenza viruses that may affect human health and timely virus sharing for risk assessment. Countries are encouraged to remain vigilant to the threat of influenza viruses and review any unusual epidemiological patterns.

WHO advises Member States to maintain surveillance for respiratory pathogens through an integrated approach, considering country context, priorities, resources and capacities. WHO has published [guidance on integrated respiratory virus surveillance](#). WHO has also updated [guidance on assessing influenza epidemic and pandemic severity](#), including the impact on healthcare facilities.

### **Clinical management and prophylaxis**

Clinical care for seasonal influenza focuses on identifying illness severity, assessing risk of progression, and linking to definitive care. Most cases are mild and self-limiting, but severe disease, marked by respiratory distress, sepsis, acute respiratory distress syndrome or multi-organ failure, requires urgent supportive care and often hospitalization. Clinical management of influenza involves high-quality supportive care—oxygen therapy, monitoring, hydration and respiratory support—and is foundational to improving outcomes, especially in severe cases.

Diagnostic testing should support rapid decision-making: nucleic acid amplification test (NAAT) is conditionally recommended for confirmation of suspected disease in severely unwell patients, while either NAAT or digital immunoassay may be used for non-severe cases, depending on context and resource availability. Testing should be performed early with the aim of identifying people in need of treatment and linking them to care, including antivirals where indicated.

Patients at high risk of progressing to severe disease are likely to benefit from antiviral to reduce their chance of admission to hospital. High-risk groups include adults  $\geq 65$  years, those with immunocompromising conditions, chronic cardiovascular, neurological or respiratory disease; malignancy, pregnancy and diabetes further elevate risk. Individuals  $\geq 85$  years or those with multiple risk factors are considered extremely high risk and might be considered for antiviral prophylaxis if exposed to influenza.

### **Infection prevention and control measures in health-care settings**

Seasonal influenza is known to cause health care-associated infection outbreaks, in particular in long-term care facilities. WHO advises the use of syndromic screening at all entry points to health-care settings and as part of daily inpatient assessment to ensure that patients with suspected or confirmed infections that are transmissible in health-care settings, including influenza, are identified as soon as possible and that appropriate transmission-based precautions are implemented. WHO advises the use of droplet precautions when caring for patients with suspected or confirmed influenza. This includes appropriate patient placement (isolation) of suspected or confirmed cases, and the use of a medical mask by all health and care workers and visitors when caring for patients with suspected or confirmed influenza.

Appropriate risk assessment for additional personal protective equipment (e.g. eye protection, filtering facepiece respirators, gown, gloves) should be performed by health and care workers when caring for patients with influenza. Increased risk of influenza transmission may occur in instances where care activities or patient symptoms are likely to generate splashes or sprays of blood, body fluids, secretions and excretions onto mucosa of eyes, nose or mouth; or if in close contact with a patient with respiratory symptoms (e.g. coughing/sneezing) and sprays of secretions may reach the mucosa of eyes, nose or mouth directly, or indirectly via contaminated hands. When performing an aerosol-generating procedure on patients with suspected or confirmed influenza, patient placement in an airborne infection isolation room as well as airborne and contact precautions with eye protection are advised.

### **Vaccination**

Vaccination is the best way to prevent influenza disease. Safe and effective vaccines have been used for more than 60 years. Influenza viruses are constantly changing, so the composition of the seasonal influenza vaccine is regularly updated to contain viruses that are more related to those circulating. WHO, through the Global Influenza Programme and GISRS, in collaboration with partners, continuously monitors influenza viruses and activity globally and recommends seasonal influenza vaccine compositions in February and September for the following northern and southern hemisphere influenza seasons, respectively.

WHO recommends annual vaccination for high-risk groups, including health and care workers. People should ideally get vaccinated just before the influenza season begins for the most effective coverage, although getting vaccinated at any time during the influenza season can still help prevent flu infections. While the effectiveness of the vaccine may vary across seasons and risk groups, it reduces disease severity and lowers the chance of complications and death. Vaccination is especially important for people at high risk of influenza complications and their caregivers.

Genetic changes or drift can occur in the circulating influenza viruses before or during the influenza season, including during the time between vaccine strain selection and the influenza season. Even if there are some genetic differences between the circulating influenza viruses and the strains that are included in the vaccines, the seasonal influenza vaccine may still provide protection against drifted viruses. Current vaccines include three influenza viruses: influenza A(H1N1)pdm09, influenza A(H3N2) and influenza B/Victoria lineage viruses. Therefore, circulation of a drifted virus does not always result in seasonal influenza vaccines being less effective in offering protection against influenza associated illness.

As of now, it remains unclear how the vaccine will protect against clinical disease during this current season. However, early vaccine effectiveness estimates show the current vaccine is 70 to 75% effective at preventing hospital attendance in children aged 2 to 17 years and 30 to 40% effective in adults.<sup>[1],[2]</sup>

### **Public health and social measures in the community**

The implementation of appropriate and proportionate public health and social measures (PHSM) is an essential component in the overall response to seasonal influenza epidemics. Measures such as performing hand hygiene, respiratory hygiene and cough etiquette as

well as voluntary self-isolation and mask wearing of individuals who are symptomatic or have tested positive for influenza viruses can reduce transmission of influenza viruses. Countries should consider developing a plan to scale up additional PHSM in the event of high or extraordinarily high epidemics.

### **Risk communication and community engagement**

Member States should consider to update and strengthen their risk communication and community engagement (RCCE) strategy integrating respiratory viruses. Enhanced risk communication and community engagement approach support empowerment of individuals to make informed decisions, countering misinformation, and community-led protection strategies.

Clear, regular, evidence-based, culturally acceptable and context adapted RCCE approaches are essential for building and maintaining trust with the concerned and affected populations to ensure adoption of interventions, practices and behaviours. For RCCE efforts to be successful, it is vital that national policies for RCCE incorporate community engagement and feedback mechanisms that acknowledge and address contextual challenges faced by different population groups, particularly those made most vulnerable. The integration of RCCE approaches to promote vaccination against influenza is also recommended.

WHO does not recommend any restriction on travel to or trade with the countries named in this report, based on the information available on the current event.